

Medicare Part D N1, N2, and N3 (Nx) Processing (TrOOP Routing)

BUSINESS REQUIREMENTS
DOCUMENT VERSION 1.6



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N1 Overview

Background

Title 1 of the Medicare Modernization Act (MMA) of 2003 establishes a new voluntary outpatient prescription drug benefit under Part D of Title XVIII of the Social Security Act that takes effect on January 1, 2006. This new drug benefit, along with an employer subsidy for qualified retiree health plans, is referred to as Medicare Part D.

The other drug coverage information supplied by the Trading Partners will enable CMS to pass along information so that pharmacies can electronically coordinate benefits in real time with other payers that provide drug coverage for Medicare beneficiaries.

Business Requirements Overview

As a part of Medicare Modernization Act (MMA) of 2003, a new entity, the TrOOP Facilitator, was established to route payment information from payer to payer and its functions were defined. The basic TrOOP Facilitation process is triggered by the submission of a transaction by a pharmacy to a secondary payer. Payment information routing from the TrOOP Facilitator to the PDP utilizes the NCPDP Information Reporting transactions, N1, N2, and N3.

Note: Unless otherwise noted, for the remainder of this document, “Nx” includes the “N1,” “N2,” and/or “N3” transaction types and “Bx” includes the “B1,” “B2,” and/or “B3” transaction types.

The primary use for the Nx transaction set is to update TrOOP balances based on information received from or being sent to secondary and later payers. This is the only function described within this document. For additional information regarding the use of Information Reporting transactions for Medicare Part D Processing, see Telecommunications Standard Implementation Guide, Version C.1, Appendix E “Use of Information Reporting (N1, N2, N3) Functionality for Medicare Part D Processing”.

These transactions include:

- Information Reporting (N1) - This transaction is used to submit supplemental payer TrOOP amounts to the primary processor.
- Information Reporting Reversal (N2) - This transaction is used to reverse a previously submitted Information Reporting transaction. It is requesting the Processor or Reporting Entity to back out the previously reported information.

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- Information Reporting Rebill (N3) - This transaction is an information reporting submission with an implied reversal. It is used by the Originator to cancel a previously submitted and processed information reporting transaction, and submit a new information reporting transaction at the same time.

CMS is working with industry to enable the Medicare plan and the third-party payer to efficiently coordinate claims in real time. That is, when the beneficiary presents his or her Medicare card at the pharmacy counter, a pharmacy will be able to query the Medicare plan's (or in some situations the TrOOP facilitator's) computer to determine whether the Part D plan or a supplemental payer should be billed for the prescription. The system will tell how much the beneficiary should pay for the prescription, and whether any third party will pay any portion of a deductible or co-insurance. All the beneficiary will see is the bottom-line result – the beneficiary deductible or co-pay (if any) that remains after the all payers have paid their parts.

As provided in the MMA, beneficiaries are legally obligated to report information about other prescription drug coverage or reimbursement for prescription drug costs that they have or expect to receive; any material misrepresentation of such information by a beneficiary may constitute grounds for termination of coverage from a Part D plan. Part D Plans must capture their enrollees other prescription drug coverage including, if known, any Rx identifiers (RxBIN, PCN, RxGRP, and RxID) – to the COB Contractor so that it can be validated, captured, and maintained in the CMS database for COB purposes.

As a PBM, when Argus processes claims as the primary payer, Argus will be required to maintain an accurate beneficiary TrOOP balance at all times. When Argus receives a Nx transaction, the beneficiary's TrOOP balance will be updated with the information from the secondary payer(s). When Argus receives a claim in a manner other than POS (e.g. paper), Argus will create a Nx transaction and forward it to the TrOOP facilitator in a batch mode. The TrOOP facilitator will forward the Nx to the primary payer so the beneficiaries' balances can be updated.

Basic Process

- This process begins after a Pharmacy has submitted a claim for a Part D Medicare Beneficiary to a Prescription Drug Plan (PDP). When applicable, the response from the PDP provides Other Payer information to the Pharmacy.
- Pharmacy submits prescription claim to Secondary Payer based upon the information received from the Primary Payer and is routed to the TrOOP Facilitator. The TrOOP Facilitator flags the supplemental request claim.
- Secondary Payer adjudicates claim.
- Secondary Payer responds to the Pharmacy via the TrOOP Facilitator.

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- TrOOP Facilitator matches the initial flagged request claim to the response claim and determines the primary PDP routing, transmits the Information Reporting transaction (N1) containing secondary Patient Paid Amount information to PDP to update TrOOP calculations, etc.
- Primary PDP applies Patient Paid Amount and adjusts the member's TrOOP by the amount paid on the member's behalf by a qualified payer.
- Primary PDP transmits response to TrOOP Facilitator.
- This process is repeated as needed for the remaining supplemental payers.

A similar process would occur for Reversals from the Pharmacy to the Other Payer, which would result in Information Reporting Reversals (N2 or N3) from the TrOOP Facilitator to the PDP. For correct TrOOP Facilitator routing environment, a unique BIN/PCN will be assigned to a secondary or later payer's health plan. This will allow the Switch to determine the appropriate routing for a Medicare patient's transactions. This will also trigger the TrOOP Facilitator to create the Nx transaction to the PDP. The unique BIN/PCN will be sent to the PDP so they may identify the correct secondary or later payer's health plan.

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Nx Requirements

General

Note: Requirements are not finalized from CMS yet. Any new requirements obtained from CMS could potentially change Argus Health Systems' proposed design solutions.

Argus Health Systems must meet the following Nx requirements in order to comply with the implementation and participation of the Medicare Part D program as outlined by the CMS business plan. Requirements for claims processing, TrOOP tracking, service billing, reporting and other aspects will be addressed in separate documents.

Nx transactions received, created, and their responses that pass basic edits must be retained and viewable via IPNS, 3270, and the Reporting Warehouse . It is expected the same screens that display the claim information can also be used to display the Nx data. Retention on these transactions should match the related claim information.

The Argus Claims processing system must:

- Recognize and accept NCPDP version C.1 Nx transactions from TrOOP Facilitator.
- Process N1, N2, and N3 as new claim types to transfer and update TrOOP amounts.

Even though Argus could be the processor for both a primary and secondary payer, we will NOT directly update information across customers, clients, or groups within a single transaction. All supplemental claims must go through the Troop Facilitator to be forwarded to CMS for audit purposes.

Receiving an Nx

The primary purpose of receiving an Nx transaction is to update the TrOOP balances with information received from secondary or later payers. Since it is not a claim transaction, functions such as DUR, pricing, RDS checking, preauth validation, and pharmacy rates are not applicable.

In order to accurately track and report on Other Health Insurance supplemental coverage information as it relates to Medicare Part D, the Argus system must capture certain data elements as specified by CMS. Some data elements already exist in the Argus system and will be utilized appropriately. In order to fully comply with Part D as it relates to NX information of members, the system must also capture:

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(These three fields could occur three times - secondary, tertiary, quaternary payers.)

- Other Payer BIN Number (990-MG) – will provide the PDP with the BIN number of the payer reporting the patient pay amount
- Other Payer Processor Control Number (991-MH)– will provide the PDP with the Processor Control Number (if used) of the payer reporting the patient pay amount
- Other Payer Group ID (992-MJ) - will provide the PDP with the Group ID, if used, of the secondary, tertiary, etc. payer

Changes to existing fields are:

- Transaction Reference Number (880-K5) – was added for use within the Telecommunication Standard and its definition modified to allow the use of this field as a tracker for the facilitator to enable them to match all claims and reversals related to a particular dispensing event. The Facilitator originates this number in the Information Reporting transactions to the PDP. The Transaction Reference Number designated in the N1 is carried through in the N2 and is reported back on the Nx response.
- Other Payer Cardholder ID (356-NU) – was modified to be included in the Request Insurance Segment of the Telecommunication Standard.

Suggested edits

Retain all transactions as received, update with the Transaction response Code and if needed, the error codes assigned during the process.

Nx specific segment and field level edits are specified in the Appendixes A, B, and C. If a mandatory field is missing or invalid, reject the transaction with a Transaction Response Status of “R” and the proper Reject Count and Reject Codes populated in the response transaction. For a list of error codes, refer to Appendix H, “VERSION 3.0 and 4.0 REJECT CODES FOR PAYMENT TAPE” in the NCPDP “Data Dictionary,” dated July 2005. Note: all possible errors should be reported, not just the first one found.

Even though processing N1 transactions as received, even if they are received out of order, could cause the Other TrOOP and PLRO to be misreported, the industry as adopted this as the best approach, The processing sequence for Other Payers is not edited, in other words, if we receive an Nx transaction from the tertiary payer before we receive the transaction from the secondary payer, an error should not be created. (See Updating Claim Information below.)

If the transaction passes the missing or invalid edits, the corresponding Bx claim should be located using logic similar to the standard Argus claim matching logic, including Customer, Client, Group, status, Pharmacy ID, NDC, Member, Date Filled, and Rx Number. If a matching claim is not found, reject

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the Nx transaction with a code 85 (Claim not processed) and a Transaction Response Status of “R”.

If a matching claim is found, Create a response and return it to the TrOOP Facilitator with a Transaction Response Status of “A”.

Before the N1 can be applied, the Patient Paid Amount Submitted should be subtracted from the Patient Paid Amount Submitted on the previous N1 or B1, giving Patient Paid Difference. Then, the COB information for the supplemental payer must be read. If the payer is a TrOOP eligible payer, the Patient Paid Difference, should be copied to the Other TrOOP Amount on the N1 claim record. If the payer is NOT a TrOOP eligible payer, the Patient Paid Difference should be copied to the PLRO field (Patient Liability Reduction by Other payer) on the N1 claim record. In either case, the deductible system should be called to update TrOOP amounts using the Patient Paid Difference field. (See Updating Claim Information samples below.)

All transactions received are retained. Those that pass the edits above, are displayed with the associated Bx transactions.

Updating the TrOOP information will move a member into and out of the Catastrophic coverage phase, often several times. Current adjustment logic for handling changes in deductible levels should be used.

At no time, should the Member deductible or TrOOP buckets become negative – if this condition occurs, reject Nx Transaction with a code DX (M/I Patient Paid Amount Submitted). Create a response and return it to the TrOOP Facilitator with a Transaction Response Status of “R”.

A duplicate Nx transaction is determined by matching using Customer, Client, Group, status, Pharmacy ID, NDC, Member, Date Filled, Rx Number, Transaction Sequence Reference Number , and Patient Paid Amount Submitted. If a duplicate Nx transaction is received, reject it with a code 83, a Transaction Response Status of “R”, but DO NOT apply the amount to TrOOP.

N2 and N3 records should be matched with the appropriate N1 record, using Customer, Client, Group, status, Pharmacy ID, NDC, Member, Date Filled, Rx Number, Transaction Sequence Reference Number, and Patient Paid Amount Submitted. If a N2 or N3 transaction is received without the related Bx or N1 transaction, reject it with a code 85, and otherwise respond with a Transaction Response Status of “A”.

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Updating the claim information

N1 Processing (correct order)

Tran type	Payer Nbr	Claim amount (Copay method)	Patient Pay	Patient Paid Difference	TrOOP Eligible	Adjustment to TrOOP Balance	TrOOP Balance Accum	Other TrOOP Xactn	PLRO Xactn
B1	1	100	45	n/a	Y	45	45	0	0
N1	2	n/a	30	15	Y	0	45	15	0
N1	3	n/a	10	20	N	-20	25	0	20

N1 Processing (out of order)

Tran type	Payer Nbr	Claim amount (Copay method)	Patient Pay	Patient Paid Difference	TrOOP Eligible	Adjustment to TrOOP Balance	TrOOP Balance Accum	Other TrOOP Xactn	PLRO Xactn
B1	1	100	45	n/a	Y	45	45	0	0
N1	3	n/a	10	35	N	-35	10	0	35
N1	2	n/a	30	-20	Y	20	30	20	0

Creating an Nx

An Nx will need to be created when Argus is the primary or supplemental payer and the claim is received NON-POS (paper, 837, or other means). This transaction is sent to the TrOOP Facilitator via a batch file. The format for the file and transaction is the same as the Nx transactions received from a POS claim and documented in Appendix A. Batch Header and Trailer formats are included in this document.

Note: At this time, the TrOOP Facilitator will accept Nx transactions in this situation only as a batch transmission.

When an Nx transaction is rejected by either the TrOOP facilitator or primary payer, the transaction should be updated with the reject reasons. A process will be developed to correct the errors and resubmit the Nx transaction.

The TrOOP Facilitator will respond to these Nx transactions with a Transaction Response code of "R" or "C" only. This is completed before the transaction is delivered to the secondary payer.

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Unresolved Requirement Issues

- Nx requirements are not complete from CMS. Argus Health Systems will update this requirements document as additional information is received from CMS.
- Requirements regarding reconciliation and adjustments due to receipt of out of order Nx transactions has not been finalized either from CMS or the industry.

Assumptions

- Records that must be retained for Medicare under Part D will be retained in the format(s) required by either state law or the HIPAA Privacy Rule, if applicable, or at the plan's discretion. CMS reserves the right to revisit this requirement in the future.
- Supplemental health insurance data will be obtained from the beneficiaries by the plan sponsors and forwarded to Argus via a set of standardized data elements.

Benefits

The addition of new fields and modification of existing fields to the Member record will allow Argus Health Systems to efficiently process customers' Medicare Part D business.

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Appendix A: Payer Sheets

Payer sheets are located at:

http://medifacd.ndchealth.com/Payers/MediFacD_Payers_ProgrammerDetails.htm#SampleTrans

or at S:\ALL\CMS Part D\Nx - TrOOP Routing
Requirements\Payers_ProgrammerDetails.pdf

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